



Community-based VIA and Thermocoagulation for Cervical Cancer Prevention in Rural Malawi

A qualitative study

Session Title: Gynecology Oncology

Fan Lee, MD
University of North Carolina Dept. Obstetrics and Gynecology





14-19 OCTUBRE 2018
RIOCENTRO | RIO DE JANEIRO | BRASIL

www.figo2018.org

Declaration of Good Standing and Conflict of Interest Disclosure

My presentation complies with FIGO's policy for declaration of good standing and conflict of interest disclosure;

I do not have a financial interest in any product or service related to my presentation;

My participation at this Congress is supported by:

- University of North Carolina Office of International affairs



14-19 OCTUBRE 2018
RIOCENTRO | RIO DE JANEIRO | BRASIL

www.figo2018.org

Learning objective

Gain insight into perspectives of women in rural Lilongwe
in regards to cervical cancer screening:

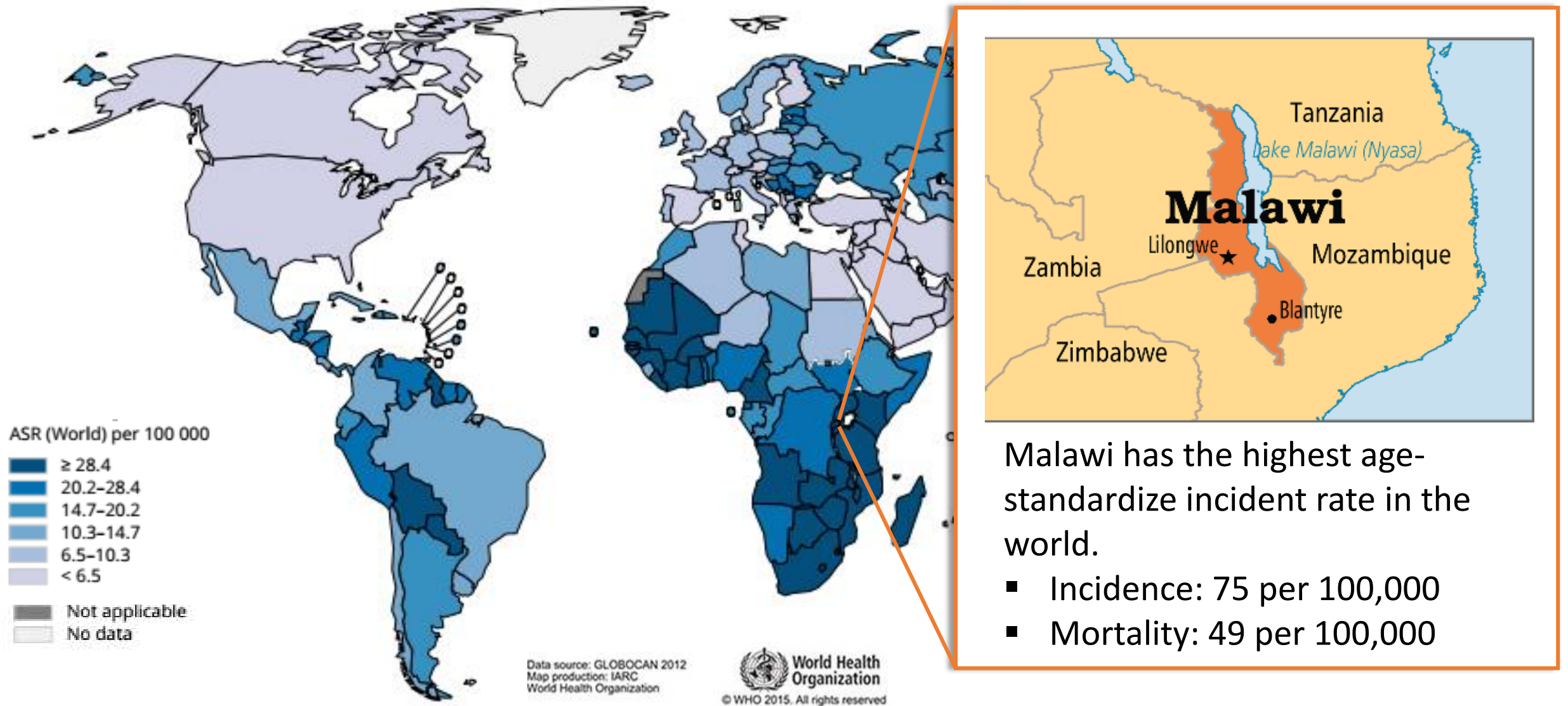
Barriers

Motivations

Experiences

BACKGROUND

Age standardized (World) incidence rates, cervix uteri, all ages



Malawi has the highest age-standardized incidence rate in the world.

- Incidence: 75 per 100,000
- Mortality: 49 per 100,000

BACKGROUND

Screen-and-treat cervical ca prevention:

Visual inspection with acetic acid (VIA) and
Cryotherapy



Limitations:

- Access to screening facilities
- Cost & availability of gas
- High lost to follow-up

Thermocoagulation:

- Heat to destroy cells
- Safety and efficacy comparable to cryotherapy

OBJECTIVES

Community-Based Cervical Cancer Screening with VIA and Thermocoagulation, a pilot program (Chinula et. al)

- To assess safety, efficacy and acceptability



Qualitative Sub-study:

- To explore the perceptions and experiences of participants
- To assess the feasibility and acceptability of this intervention

METHODS



- July – August, 2017 in rural Lilongwe District
- 415 women screened with VIA
 - 29 women treated with Thermocoagulation

- 17 in-depth interviews, semi-structured
- Audiotaped, translated & transcribed
- Content analysis
- NVIVO
- Constant comparative analysis



RESULTS: DEMOGRAPHICS

Participant Demographics (N=17)		
Age		% (n)
	20's	12% (2)
	30's	47% (8)
	40's	35% (6)
	50's	6% (1)
Level of education		
	no formal	24% (4)
	Some Primary	35% (6)
	Completed Primary	29% (5)
	Some Secondary	12% (2)
Marital status		
	Married monogamous	65% (11)
	Married polygamous	24% (4)
	other	12% (2)
Partners with additional partners		
	yes	65% (11)
	no	12% (2)
	unknown	24% (4)
Total lifetime partners		
	1 partner	35% (6)
	2-3 partners	59% (10)
	≥4 partners	6% (1)
HIV status		
	negative	100% (17)



RESULTS: KNOWLEDGE & PERCEPTION

- There was low baseline knowledge, fatalistic views of cervical cancer and myths/misconception of screening.

“Cervical cancer is a very deadly disease, everyone is afraid of it more than AIDS” (ID233, age 40)

- Participants commonly expressed desire to “know one’s status,” as the main reason for partaking in screening.

“I wanted to know the condition of my body. you can just be staying and never be certain that you are ok or not. So this time I thought it wise to go and get screened” (ID269, age 26)

RESULTS: EXPERIENCE OF SCREEN & TREAT

➤ All participants expressed an overwhelmingly positive experience

Appreciations:

- Healthcare workers coming to village
- Felt well counseled
- Immediate treatment offered
- Comfortable during procedure

Concerns:

- Noisy equipment
- Long duration
- Worry of positive result
- Vaginal discharge

RESULTS: BARRIERS

- Transportation – expressed by EVERY participant
- Male partners were viewed as both a barrier AND as an important source of support

*“After I told him [about the screening], he did not understand....We managed to stay for 2 weeks without sex but that was because he was angry. The third week however things got worse and then we had sex. So it only worked for 2 weeks and that was because he was angry.”
(ID329, age 41)*

DISCUSSION / KEY MESSAGES

- Community-based, screen-and-treat with Thermocoagulation was widely accepted by participants
- Transportation is a major barrier to follow-up
- Thorough counseling and positive experience with healthcare providers helped participants understand the purpose and importance of screening
- Education campaigns should also target men to promote male partner support
- Future considerations for reaching rural women should include self collected vaginal swabs for HPV testing

THANKYOU

Participants of this study
UNC Project Malawi and study staff
Lilongwe District Health Office

Mentors:
Drs: Jennifer Tang, Lameck Chinula, Benjamin Chi, Jes Morse

Study Team:
Agatha Bula, John Chopola, Clement Mpange, Laura Limarzi

UNC Office of International Affairs
UNC OBGYN Residency
FIGO

REFERENCES

1. Globocan 2012: Estimated Cancer Incidence, Mortality and Prevalence Worldwide in 2012
http://globocan.iarc.fr/Pages/fact_sheets_population.aspx
2. Ferlay J SI, Ervik M, Dikshit R, Eser S, Mathers C, Rebelo M, Parkin DM, Forman D, Bray F.GLOBOCAN 2012 v1.0, Cancer Incidence and Mortality Worldwide: IARC CancerBase No. 11. Lyon, France: International Agency for Research on Cancer; 2013.
3. Maseko, F. C., Chirwa, M. L., & Muula, A. S. (2014). Client satisfaction with cervical cancer screening in Malawi. *BMC health services research*, 14(1), 420.
4. Campbell, C., Kafwafwa, S., Brown, H., Walker, G., Madetsa, B., Deeny, M., ... & Cubie, H. A. (2016). Use of thermo-coagulation as an alternative treatment modality in a 'screen-and-treat' programme of cervical screening in rural Malawi. *International journal of cancer*, 139(4), 908-915.
5. Msyamboza, K. P., Phiri, T., Sichali, W., Kwenda, W., & Kachale, F. (2016). Cervical cancer screening uptake and challenges in Malawi from 2011 to 2015: retrospective cohort study. *BMC Public Health*, 16(1), 806.
6. Dolman, L., Sauvaget, C., Muwonge, R., & Sankaranarayanan, R. (2014). Meta-analysis of the efficacy of cold coagulation as a treatment method for cervical intraepithelial neoplasia: a systematic review. *BJOG: An International Journal of Obstetrics & Gynaecology*, 121(8), 929-942.
7. Sauvaget, C., Muwonge, R., & Sankaranarayanan, R. (2013). Meta-analysis of the effectiveness of cryotherapy in the treatment of cervical intraepithelial neoplasia. *International Journal of Gynecology & Obstetrics*, 120(3), 218-223.
8. Viviano, M., Kenfack, B., Catarino, R., Tincho, E., Temogne, L., Benski, A. C., ... & Petignat, P. (2017). Feasibility of thermocoagulation in a screen-and-treat approach for the treatment of cervical precancerous lesions in sub-Saharan Africa. *BMC women's health*, 17(1), 2.

QUESTIONS?

“I will use my story and experience as a tool to encourage my fellow women to go for cervical cancer screening, because I am a living example.” -ID240, age 36



14-19 OCTUBRE 2018
RIOCENTRO | RIO DE JANEIRO | BRASIL

www.figo2018.org